

Public Health Emergency Operations Center
"COUSP DRC"

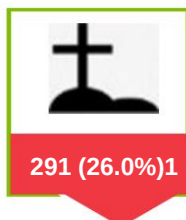
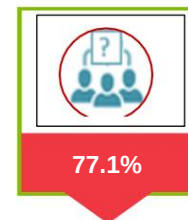
MVE-17 Incident Management System

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Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°040/MVB_23/06/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (34)	- Ituri (22/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia and Fataki. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	June 23, 2026
Publication date	June 24, 2026

Total confirmed cases
for the 3 provincesCumulative deaths
among confirmed cases
and case fatality ratePatients in
isolation -
hospitalizationCumulative number
of recoveriesSuspected cases of
the dayContact tracing rates
for the 3 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

Twenty **-four (24) new confirmed cases, including 12 deaths**, were reported at the end of the day on June 23, 2026, in the provinces of **Ituri (23, including 12 deaths) and North Kivu (1)**. These confirmed cases are distributed across the health zones of **Bunia (8), Mongbwalu (4), Komanda (3), Mangala (3), Nia-Nia (3), and Lita (2)** in the province of Ituri and **Butembo (1)** in the province of North Kivu.

- **Seven (7) patients** with Ebola virus disease in **Ituri** have been declared cured after obtaining negative control tests.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

The Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For more than two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is

The population of the region is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, the Ituri province had already been affected by the 10th Ebola virus (EV) epidemic, which was then raging in North Kivu. The current epidemic, caused by Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

1 2 additional deaths recorded in North Kivu among previously confirmed cases.

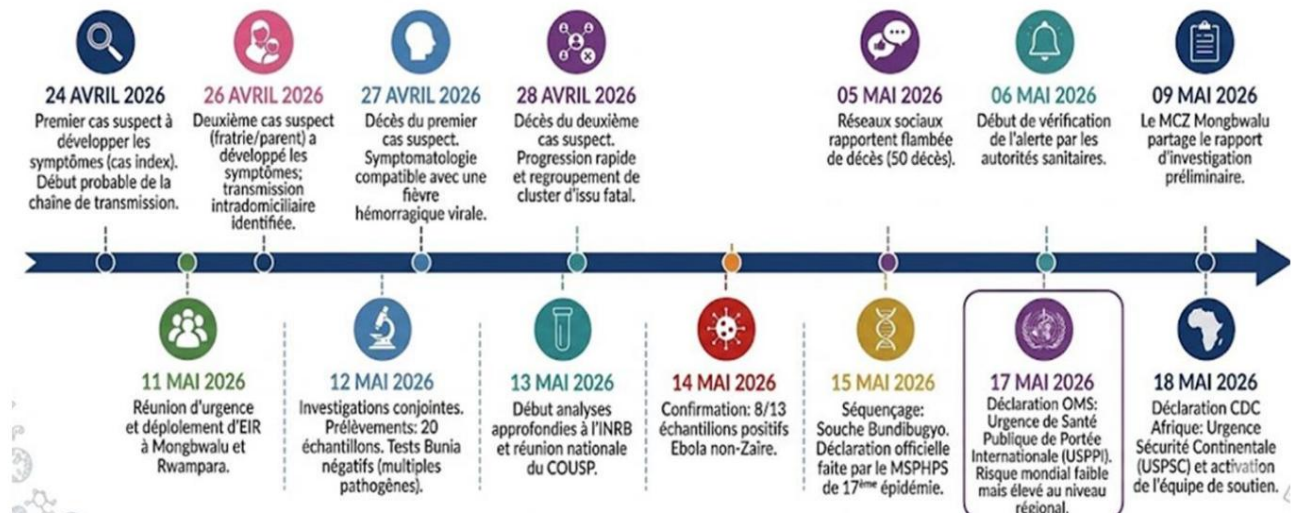


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 23, 2016

Table 1. Distribution of confirmed cases and deaths by affected province as of June 23, 2016.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	1020	238	23.3%	22 out of 36 (61.1%)	23
North Kivu	95	52*	54.7%	11 out of 34 (32.3%)	1
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	1118	291	26.0%	34 out of 104 (32.7%)	24

* confirmed cases (1 in Kyondo and 1 in Musienne).

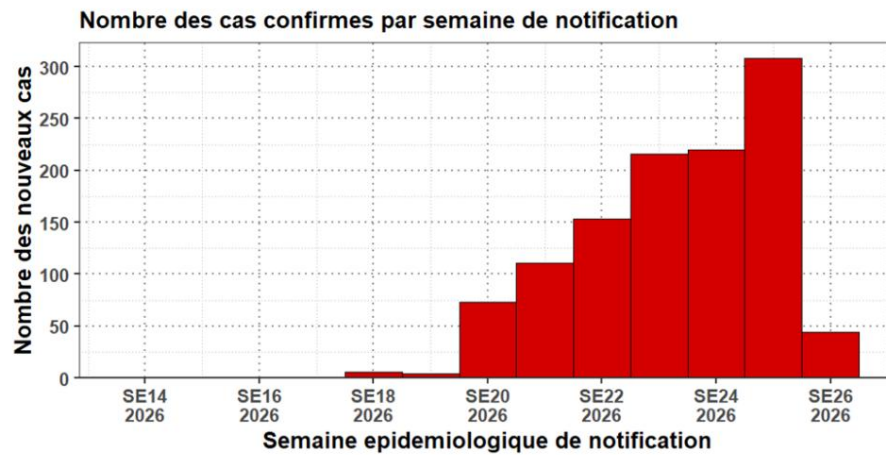
No new health zones were affected by the end of June 23, 2016. The number of affected health zones remains at 34/104 (section 3.1). Since May 26, 2016, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission. However, active surveillance is ongoing. remains necessary.

3.2 Distribution by health zone (Cumulative total as of 23/06/2016)

- **Province of Ituri (1,020 cases):** Bunia (304), Mongbwalu (241), Rwampara (225), Nyankunde (86), Nizi (30), Lita (26), Mangala (17), Bambu (14), Komanda (13), Tchomia (8), Kilo (7), Aungba (6), Logo (5), Nia-Nia (5), Damascus (4), Rimba (3), Aru (3), Mambasa (2), Drodoro (1), Gety (1), Kambala (1), and Fataki (1). Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.
- **North Kivu Province (95 cases):** Katwa (32), Butembo (29), Beni (20), Oicha (3), Kyondo (3), Kalunguta (2), Musienene (2), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).
- **South Kivu Province (3 cases):** Miti-Murhesa (3).

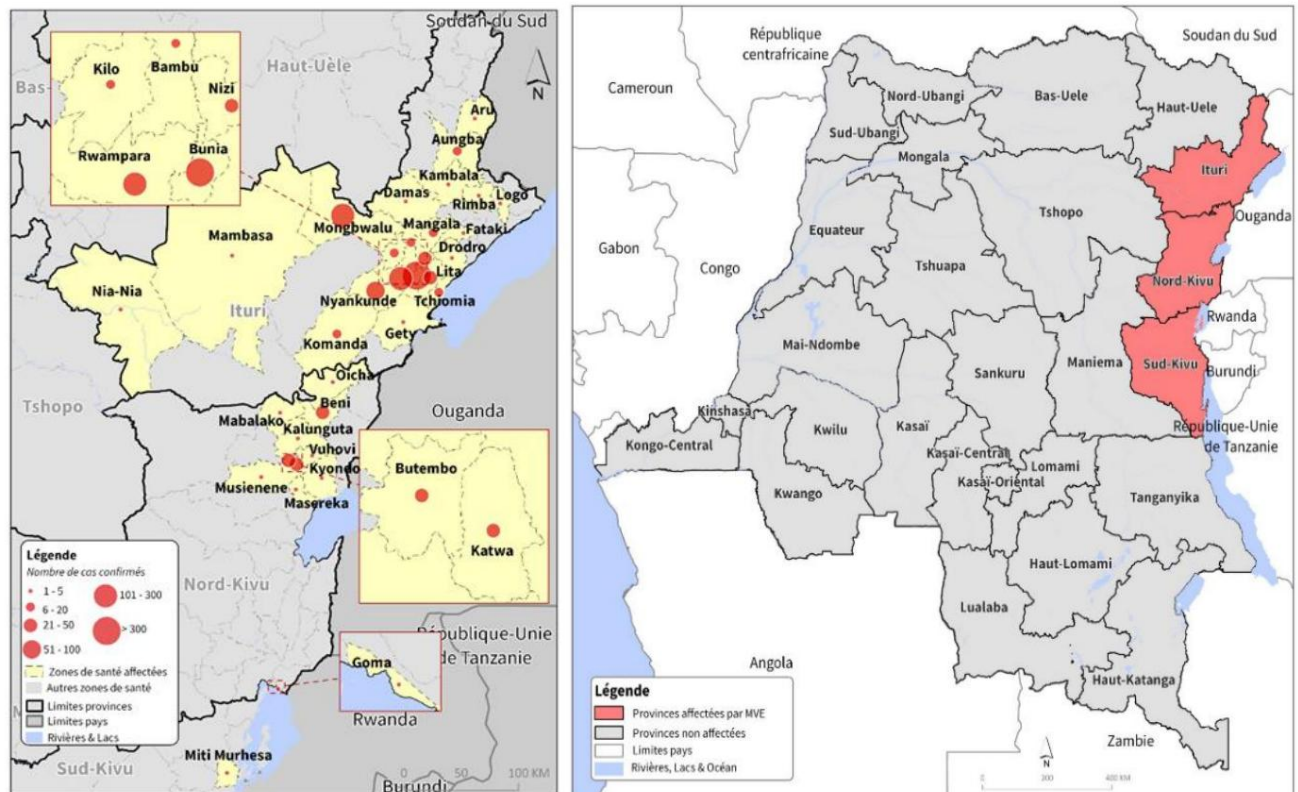
3.3 Temporal analysis: We observe an increasing number of confirmed cases from one week to the next, reflecting continuous transmission of the disease in the community even during the epidemiological weeks

On weeks 23 and 24, the number of cases appeared to be stable. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capacities) contributed to early detection, confirming continued and increased community transmission in week 25. Given that week 26/2026 is not yet over, we cannot compare it to previous weeks.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 1135.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 23, 2026

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (June 23, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR, %)
ITURI	1020	238	23.3%
Bunia	304	54	17.8%
Rwampara	225	37	16.4%
Mongbwalu	241	103	42.7%
Nyankunde	86	9	10.5%
Nizi	30	6	20.0%
Lita	26	6	23.1%
Komanda	13	3	23.1%
Bamboo	14	2	14.3%
Kilo	7		14.3%
Mangala	17	1	47.1%
Tchomia	8	8 0	0.0%
Damascus	4	0	0.0%
Aungba	6	2	33.3%
Rimba	3	0	0.0%
Aru	3		33.3%
Mambasa	2	1 1	50.0%
Logo	5	0	0.0%
Drodro	1		100.0%
Gety	1	1	0.0%
Kambala	1	0 1	100.0%
Nia-Nia	5		60.0%
Fataki	1	30	0.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	95	52	54.7%
Butembo	29	13	44.8%
Katwa	32	19	59.4%
Blessed	20	12	60.0%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	3	1	66.7%
Goma	1	2	0.0%
Masereka	1	0	0.0%
Vuhovi	1		100.0%
Mabalako	1	0	0.0%
Musienene	2	1 0 2	100.0%
SOUTH KIVU	3	1	33.3%
Miti-Murhesa	3	1	33.3%
TOTAL 291	1118		26.0%

The epidemiological situation of the 17th Ebola Virus Disease (EVD) outbreak in the Republic

The Democratic Republic of Congo continues its spread, with 24 new confirmed cases, including 12 deaths .

recorded in the last 24 hours, bringing the total to **1,118 confirmed cases** and **291 deaths**, expressing an overall case fatality rate of **26.0%**.

Provincial dynamics and case concentration

Approximately **91.2%** of cases (n=1,020) and **81.8%** of deaths (n=238), representing a case fatality rate of **23.3%**, are concentrated in **Ituri** province, making it the absolute epicenter of the epidemic. The health zones of Bunia (304 cases), Mongbwalu (241 cases), and Rwampara (225 cases) account for 75.5% of all reported cases in this province, and are therefore the most affected. Furthermore, the health zones of Bunia and Rwampara have moderate case fatality rates (17.8% and 16.4% respectively), while the **Mongbwalu** zone shows marked severity, with a case fatality rate of **42.7%** (103 deaths out of 241 cases), suggesting persistent challenges in early intervention or access to care. It should be noted that 17 additional cases (with no associated deaths) cases have been reported, while the health zones of origin remain to be identified, which could indicate a geographical spread requiring further investigation.

Severity of the epidemic in North Kivu

A total of **95 confirmed cases** and **52 deaths** have been reported in this province, representing a case fatality rate of 54.7%. This last one presents the most worrying profile in terms of severity, higher than the national average. This high mortality rate is driven by the health zones of **Beni** (60.0%), **Katwa** (59.4%) and **Oicha** (66.7%). The Katwa health zone had the highest number of reported cases (n=32), followed by the Butembo health zone (n=29) with a lower case fatality rate (44.8%). A case fatality rate of 100% was exceptionally observed in the Vuhovi zone (with a small sample size), highlighting the danger posed by the virus in this region, and the need for an immediate clinical response.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Supervision of field teams and the various patient care structures.
- Holding the coordination meeting and supporting the pillars of the response.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 23, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Report (D-1)		31	0	13	44
New Alerts		129	515	11	655
Total alerts for the day		160	515	24	699
Alerts investigated		160	515	20	695
Investigation rate		100.0%	100.0%	83.3%	99.4%
Alerts confirmed	Alive	47	39	7	93
	Deceased	19	26	0	45
Total suspected cases for today		66	65	7	138

During the course of June 23, 2026, **699 alerts** were reported across the three provinces, **695 (99.4%)** were investigated, **138 suspected cases including 45 deaths** were identified (Table 3).

Table 4. Contact tracing of confirmed cases as of June 23, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	6,312	4,918	77.9%
North Kivu	1938	1,421	73.3%
South Kivu	96	96	100.0%
Total	8,346	6,435	77.1%

**The follow-up rate in North Kivu province was calculated based on the health zones that reported contact tracing data (completeness: 7/10: 70.0%).*

- Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and followed per case).
- **307 new contacts** were identified in 5 health zones in Ituri province. Furthermore, **155 contacts successfully completed their 21-day follow-up, including 78 in South Kivu and 77 in North Kivu.**

Update on checkpoint and point of entry (PoC/PoE) activities

- **In Ituri :** Four (4) alerts were notified to the PoE/PoC, including 1 (alive) validated and referred to the Bunia General Hospital from the Mudzibala PoC, one lifeless body intercepted and swabé at the Mudzibala PoC, 1 alert (alive) invalidated at PoC FONER Mambasa and 1 alert (alive) observed then released at PoC Ombay/ZS Ariwara.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of June 23, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	85.5% (47/55)	92.3% (12/13)	86.8% (59/68)
Number of people who went through PoE/PoC	72,530	150,276	222,806
testing, % of travelers screened,	99.7%	97.5%	97.5%
% of travelers who washed their hands, %	97.7%	97.6%	97.7%
of travelers made aware of the issue	94.6%	97.0%	96.6%
Number of alerts notified	4	0	4
Number of validated live alerts	1	0	1
Number of lifeless bodies intercepted today	1	0	1
Number of problematic contacts intercepted today; %	0	0	0
of validated active alerts referred to CT/CTE	100.0%	0.0%	100.0%
% of lifeless bodies swabé	100.0%	0.0%	100.0%
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri:** 74 samples collected and analyzed (positivity 31.1%; n=23).
- **North Kivu :** 76 new samples collected, of which 39 were analyzed and 1 came back positive for Ebola virus disease (positivity 2.6%).
- **South Kivu :** 7 samples were collected and analyzed, all came back negative.

4.4. Infection Prevention and Control (IPC)

Ituri: Completion of 25/29 safe and dignified burials (EDS), with 4 postponements; decontamination of 10 households and 4 social service establishments (ESS) in the Bunia and Rwampara health zones; briefings and support provided. Infection prevention and control (IPC) carried out with 85 service providers in the health zones of Bunia, Rwampara, Komanda and Nia-Nia; decontamination of a septic system in Mongbwalu after the transfer of suspected cases; provision of kits and supplies

PCI/WASH in Mambasa and Mongbwalu; 4 ESS and 1 church decontaminated around confirmed cases; PCI briefings and support carried out in Bunia, Mongbwalu, Bambu and Mambasa.

North Kivu : Briefing of 290 healthcare providers from the health zones of Beni, Butembo, Katwa, Oicha, Goma, Karisimbi and Nyiragongo on various IPC themes in the context of the Ebola epidemic; evaluation of 11 health facilities including 6 in Beni, 3 in Butembo, 1 in Oicha and 1 in Katwa (The average score is 45.8%); closing of the training of 34 providers from 17 priority health facilities in the Karisimbi health zone with the support of IMA; decontamination of 4 health facilities including 2 in Katwa and 2 in Beni around confirmed cases; decontamination of a household in the Kihinga health area around the confirmed case in the Katwa health zone; provision of IPC supplies to the Mukuna Hospital and Jerusalem Hospital in the Katwa health zone as well as the General Referral Hospital in the Oicha health zone; Over the past 24 hours, 26 death alerts were processed at the morgue and 24 post-mortem samples were taken before burial. However, only 5 EDS (Extracorporeal Diagnosis) tests were performed.

South Kivu : Distribution of IPC and PPE inputs in 5 health centers in the Katana health zone with support from WHO; a workshop was organized with simulation exercises (donning and doffing of PPE, EDS); coaching on the use of the Kobocollect tool for conducting IPC scorecard assessments for the 45 members of the IPC pillar in Rio; bio-cleaning of the 17 schools selected as state examination centers; 4 scorecard assessments carried out, including 2 in the Idjwi health zone (Nyakaingwa Hospital: 56.5%, Kisiza Health Center: 24.2%), 1 in the Fizi health zone (Mshimbakye Hospital: 43.5%), and 1 in the Nundu health zone (Nundu General Hospital: 32.3%); installation of 4 Points of Control (PoCs), including 3 in the Lwiro health area and 1 in the Kabushwa health area, with support from PNHF/IOM.

4.5 Holistic care

- **Seven (7) patients** were declared cured in Ituri, after obtaining negative control results.

Thus, the cumulative number of recovered patients is **122, including 113 in Ituri, 7 in North Kivu and 2 in South Kivu.**

- Two (2) recovered PPLs were readmitted to the Rwampara CTE, one of whom became positive again.
- Continued medical care, transfers to CTE/CT facilities, notification of results, and nutritional monitoring of patients. In total, 724 hot meals were served, including 163 to confirmed cases, 146 to suspected cases, 353 to PPL (People with Disabilities) and 62 to accompanying persons.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 23, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	321	56	10	387
Total admissions (24h)	46	8	7	61
Exits (24h)				
Deceased (Suspects/Confessions)	9	0	0	9
No case	14	5	5	24
Cured	7	0	0	7
Escapees (Suspects/Confessions)	0	0	0	0
Transferred to the HGR	0	0	0	0
Total outflows	30	5	5	40
Patients in isolation (End of Day),	337	59	12	408
including confirmed cases	175	17	0	192
(NC+AC) and suspected cases	162	42	12	216
Overall occupancy rate	91.3%	57.3%	37.5%	81.0%

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 23, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	65 (100.0%)	1 (100.0%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	51 (47.7%)	12 (100.0%)	0
New suspected cases that benefited from SMSPS interventions	7 (26.9%)	6 (100.0%)	0
Suspected cases under follow-up that have received interventions SMSPS	56 (43.1%)	16 (100.0%)	3 (100.0%)
Laboratory results have been announced.	26	6	0
Including positive ones	16	1	0
Number of children separated in daycare who benefited from interventions SMSPS	2 (100.0%)	0 (0.0%)	1 (100.0%)
Number of children separated in the community who have benefited from interventions SMSPS	0	0	31
Number of accompanying persons at the CTE who benefited from SMSPS interventions	34	15	4
Number of PPLs who benefited from SMSPS interventions	7	6	22
Number of household and community members who benefited from SMSPS interventions	8	32	0
Number of EDS supported	0	4	0

4.6 Continuity of care

- Monitoring of the Rwampara CTE expansion works, progressive improvement of the technical platform and continuation of patient transfers from health centers/communities to the selected treatment sites

4.7 Risk communication and community engagement

Ituri: 97 mass awareness activities were reported, reaching 4,333 people; 1,587 households were visited and 375 educational talks reached 9,375 people. The activities focused on prevention measures, reporting alerts, the importance of EDS, decontamination, and contact tracing; 1,587 home visits by community health workers reached 6,392 people (2,882 men and 3,510 women) in the health zones of Mongbwalu, Rwampara, and Nyankunde, focusing on Ebola virus disease prevention measures and reporting of deaths and survivors; Educational talks were held in the health zones of Rethy and Rwampara, reaching 2,983 people (1,219 men and 1,764 women) on prevention measures and the benefits of early access to an Ebola Treatment Center (ETC). 18 broadcasts of the spots and message on preventive measures for Ebola Virus Disease.

Support for the pillars of the response : Surveillance: Listing, investigation, and contact tracing in the Rwampara DELE health center and the CTE CME treatment center. **Care :** Support for the admission of 4 people to the CTE (3 men and 1 woman); raising awareness among a family regarding referral to the CTE for a patient presenting symptoms (LENGABO health center). Infection Prevention and **Control (IPC):** Decontamination (HOHO health center, Rwampara health zone); preparing 11 families to participate in the EDS (Rwampara health zone). **Outcome :** EDS completed

North Kivu: 38,541 people reached (22,274 women and 16,267 men), including an additional 1,248 people (318 men and 854 women) reached during home visits in the Goma, Karisimbi, and Nyiragongo health zones. These activities were conducted by 2,077 community health workers (CHWs) who visited 11,544 households; 2,159 people reached in public places (markets, parking lots, and community spaces) thanks to Red Cross volunteers, including 15 members of the Katsya health area community; An interactive program broadcast on RTCT also generated 143 community responses (calls and SMS messages), reflecting strong community engagement. 122 community actors (82 capacity-building participants and 40 participants in community dialogue, including PRESICODESA and local leaders) were engaged in Karisimbi to strengthen Ebola prevention, alert reporting and the management of resistance

; 88 community alerts reported, accompanied by 3 rumors collected and clarified, contributing to improved understanding of the disease and reducing misinformation; 18 committed local radio stations ensured the production and broadcast of 3 communication formats (interactive programs, radio spots, and microprograms); 93 teachers and school principals mobilized in the Ebola response during the State Examinations helped raise awareness among 2,175 students about preventive measures, particularly hand hygiene.
school environment.

Infodemic monitoring : 211 feedback messages were collected via community channels, radio, call centers, and online platforms, with call centers being the most prevalent (58%). The analysis highlights the need to strengthen community listening, address rumors, and disseminate testimonies from those who have recovered, particularly in light of community concerns raised in Katwa, North Kivu.

4.8. Logistics

- In total: 28 vehicles and 9 ambulances (PNC, FARDC) were made available to the various pillars to ensure their mobility towards the main roads.
- Delivery of UNICEF medicines and supplies to the LITA health zone
- Additional loading of PCI and PEC inputs for the LOGO ZS
- Supply of PCI inputs for the "Well-being" pillar.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Continued briefings for stakeholders, community awareness-raising on exploitation, abuse and sexual harassment, as well as the signing of codes of good conduct.
- **South Kivu :** Briefing of 46 RECO and community leaders in the ZS of MITIMURHESA, AS of Saint SPX, including 23F and signing of the code of good conduct.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity in CTE and saturation in Ituri (91.3%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3 Poor performance in contact tracing (77.1% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not localized or unclassified (17 cases)	Major gaps in the reconstruction of transmission chains	Conducting a data reconciliation, Remove any possible duplicates and reclassify. or specify the health zone and status (deceased, recovered, active, etc.). Active verification on the ground.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (78 cases) HCW (including 18 deaths, case fatality rate 23.1%)	Urgent supply, strengthening of IPC training
8 Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (54.7%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10. Insufficient coordination and low utilization of COUSP frameworks	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13 Continuity of essential services (CEHS) compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

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